RURAL HEALTH TRANSFORMATION · 50-STATE VISUAL SYNTHESIS

The backbone is broad. The care connection is the next test.

50
STATE PROPOSALS

Across 2025 state proposal abstracts, care redesign and digital ambition are common. Explicit technology links to named care workflows are less so.

By Martin Blanco · github.com/mbvibecode · Prepared October 2026.

01

BUILD THE CARE BACKBONE

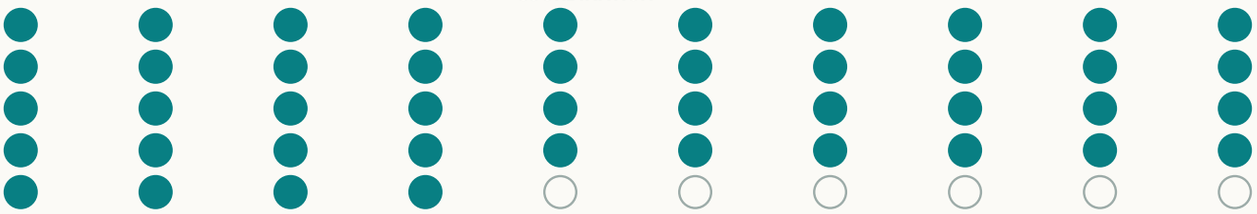
Care redesign clusters around three shared priorities.

50
WORKFORCE

47
PREVENTION / CHRONIC

47
NETWORKS / COORDINATION

44 / 50
NAME ALL THREE



Each circle = one state · Filled: names all three · Outline: does not

02

ADD THE DIGITAL RAILS

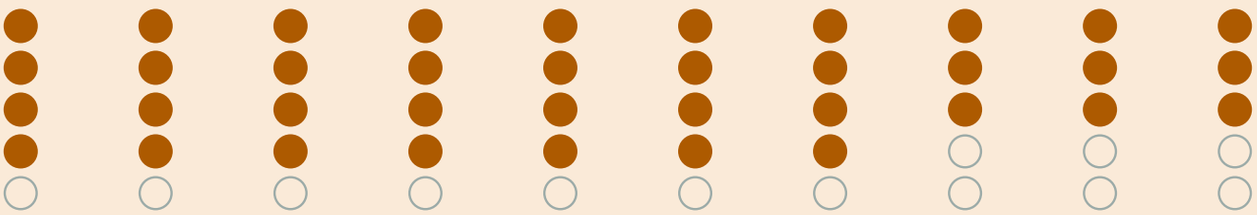
Digital ambition is broad — and frequently paired.

46
ANY TECH CATEGORY

43
DATA / RECORDS

40
TELEHEALTH / REMOTE

37 / 50
NAME DATA + TELEHEALTH



Each circle = one state · Filled: names data + telehealth · Outline: does not

03

MAKE THE CONNECTION EXPLICIT

Technology is common. Care-area linkage is less explicit.

All 34 states naming behavioral health and all 11 naming home/LTC also mention technology somewhere in the abstract. Far fewer explicitly connect each care area to technology.

BEHAVIORAL HEALTH

10 / 34 29%



explicitly link the care area and technology

HOME / LONG-TERM CARE

3 / 11 27%



explicitly link the care area and technology

explicit care + technology link

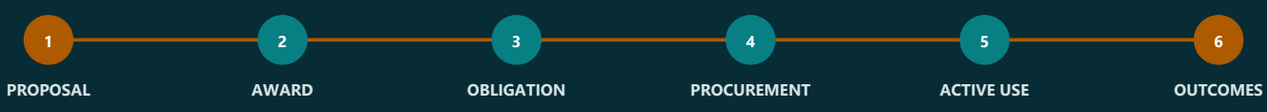
care area named; explicit link not found

These are proposal mentions — not confirmed workflows, services, or outcomes.

04

FOLLOW THE IMPLEMENTATION CHAIN

Make the connection visible after the proposal.



Measure reach, exchange use, referral closure, and continuity — not purchases alone.

METHODS Analyst-curated first-pass coding of 50 December 2025 state abstracts; no independent second-coder validation. Categories overlap. Mentions do not establish priority, spending, implementation, or performance.

Explore the state evidence: mbvibecode.github.io/rht-state-atlas/

RHT INFOGRAPHIC